

A Brief Guideline for iDEC Teams Carrying Out Inclusivity Work on Biomedical Projects

A practical, no-nonsense guide for teams building diagnostic, therapeutic, probiotic, biosensor, or other healthcare-oriented projects — written by a team that has been there.

LZU-CHINA · GutSentry · Open Community Resource · Team Guideline

Who this is for

You are an iDEC team. Your project is biomedical — a diagnostic, a therapeutic, a probiotic, a biosensor, something that will one day meet a human being. You know you should do something about "inclusivity" for your Human Practices, but you are not sure what counts, how much is enough, or how to avoid it looking like an afterthought.

This guideline is the guide we wish we had had at the start of our season. It is short on theory and long on checklists.

NOTE · Written by LZU-CHINA (2026) for the GutSentry project — an EcN-based smart diagnostic-therapeutic system for perioperative gut infections. What worked for us may not work for you; adapt freely.

1 · Start with one sentence

Before you do anything else, answer this in one sentence:

"Our project is meant to help [who], but it might leave out [whom] because [what]."

If you cannot answer that, do not run any events yet. Go and read a few papers and talk to one person. The rest of this guideline follows from that one sentence.

For us it was: "Our project is meant to help patients at risk of gut infection, but it might leave out elderly patients, people in small hospitals, visually impaired users, and low-income groups because the output is a colour and the workflow assumes a well-equipped lab."

2 · Map your stakeholders (do not stop at "patients")

Most teams write "patients" on their stakeholder list and stop. Patients are hard to reach ethically, and they are only one voice. Map at least four groups:

Group	Examples	Why they matter
End users	Patients, elderly people, people with disabilities, rural residents	They live with the problem your project addresses
Front-line professionals	Nurses, lab technicians, community health workers, paramedics	They know where a clever device fails in practice
Proxy advocates	Disability NGOs, patient associations, Red Cross branches, carers	They speak with — not for — excluded groups
Domain experts	Clinicians, microbiologists, public-health researchers, bioethicists	They keep your claims honest and clinically grounded

Aim for a mix — at least one person from each of the first three groups. Five to eight conversations is plenty; twenty is not better.

3 · Talk to people before you build

This is the single most important rule, and the one teams break most often. Do not finish your circuit and then go looking for quotes to validate it. Go talk first.

- Use the semi-structured interview guides in our companion document (Part 1 of this resource pack).
- Ask open questions: "What would go wrong?" beats "Do you like our idea?"
- Follow up every answer with "Can you give me an example?" — that is where specifics live.
- Take an easy-read information sheet and a short consent form. It takes ten minutes and it is the right thing to do.

- Record notes within 48 hours and send a summary back for correction.

NOTE · If direct patient interviews are not feasible for your team — clinical ethics, consent capacity, ward restrictions — use proxy stakeholders (nurses, NGOs, family carers) and triangulate their input against published clinical literature. State this openly. It is more credible than pretending otherwise.

4 · Turn feedback into design decisions (the bit judges read)

This is where most teams lose marks. Collecting feedback is not enough — you have to show what you changed because of it. Use the analysis matrix in our templates document, then write it up as a short table:

Stakeholder said	We did about it	Where it lives in our project
"Wards are dark, colours are hard to read"	Specified a high-contrast chromogen as default	Design / Inclusivity
"No big machines here"	Kept the whole workflow instrument-free	Design / Inclusivity
"A colour alone leaves people out"	Added tactile and odour-based readout as an optional module	Design / Inclusivity
"Simpler is better"	Cut the sample prep to a single step	Design / Inclusivity
"Braille would help"	Left space for a braille overlay on all printed materials	Education / Inclusivity

Three rules that will save you grief:

1. One feedback item = one design decision. If you cannot name the decision, you are still at the outreach stage — move that content to Education.
2. Be honest about what you did not change. "We did not alter the wet-lab prototype; we changed our translational design" is a perfectly valid answer and shows maturity.
3. State your limitations. What you could not do, and what you plan to do next, belongs in your Reflection section — not hidden.

5 · Do not confuse Inclusivity, Human Practices, and Education

These three get mixed up constantly, and it weakens your Wiki. Keep them separate:

Section	Answers the question	Typical content
Integrated Human	How did stakeholder input	Feedback → design decisions;

Practices	change your project?	ethics; expert review
Inclusivity	Who gets left out, and how are you making sure they are not?	Accessibility; equity; marginalised-group engagement
Education / Public Engagement	How are you sharing knowledge with the public?	School lectures; booths; workshops; materials

A booth where you teach 300 people is Education. An interview where a nurse tells you the ward lighting is poor and you change your chromogen choice is Inclusivity feeding into Human Practices. Put each piece of evidence in the right drawer.

6 · Make something reusable

This guide encourages reusable outputs. Pick two or three of the following and actually finish them:

- Your interview guides (stripped of project-specific detail)
- A blank stakeholder-feedback analysis matrix
- A plain-language handbook or poster set (with source files, not just PDFs)
- An audio version of your project explainer
- A short "lessons learned" page with what you would do differently

NOTE · A note on source files: a PDF of your poster is not a source file. A Canva/PowerPoint /Inkscape project file, or at minimum an editable document with the text and image licences listed, is. Judges and future teams need to be able to remix it.

7 · A realistic timeline

Stage	What to do	Roughly when
Month 1–2	Read papers, write your one-sentence gap, identify 4 stakeholder groups	Early season
Month 2–3	Run 5–8 interviews; get consent right; take notes within 48 h	Before major design freeze
Month 3–4	Build your analysis matrix; decide what changes; update Design page	Mid season
Month 4–5	Run workshops / lectures / booths using the materials you made	After you know your project
Month 5–6	Collect feedback on the events; write Reflection; package reusable assets	Late season

Wiki freeze	Three clear pages: Inclusivity, Integrated HP, Education — no duplicate content	Before deadline
-------------	---	-----------------

8 · Common mistakes (we made some of these)

- Writing "we will consider X in future iterations" instead of "we specified X". Judges read right through that.
- Using the same generic template for every stakeholder. Tailor your questions — it shows you listened.
- Only interviewing friendly academics. You learn the least from people who already agree with you.
- Counting event attendance as impact. Numbers are fine as context; they are not outcomes.
- Putting every activity in one giant "HP" page. Split it. Judges navigate by section.
- Claiming you did more than you did. Name what you could not do — it is a strength, not a weakness.

9 · A short checklist for your Wiki

4. One clear sentence stating whose needs your project might overlook
5. At least 3 distinct stakeholder groups, with evidence of real conversations
6. An ethics note: information sheet, consent, anonymisation, data handling
7. A feedback-to-decision table, linked from your Design page
8. One honest paragraph on limitations and what you could not do
9. At least two genuinely reusable, editable assets
10. Separate Inclusivity / Integrated HP / Education pages with cross-links
11. Attribution for any images, quotes, or materials you did not create yourself

That is the whole thing. Go talk to people, change something because of what they said, write it down clearly, and put your materials where someone else can use them. Good luck — and remember, the goal is not a perfect project. It is a project that took the people it affects seriously.